350 Fifth Avenue, 34th Floor
New York, NY 10118-3299
Tel: +1-212-290-4700
Fax: +1-212-736-1300; 917-591-3452

HUMAN RIGHTS WATCH

HRW.org

September 8, 2023

Submitted via Regulations.gov

Comment Intake
Request for Information Regarding Medical Payment Products
Consumer Financial Protection Bureau
1700 G Street NW
Washington, DC 20552

**Re: Request for Information Regarding Medical Payment Products**
**Docket Numbers CFPB–2023–0038; CMS–2023–0106; TREAS–DO–2023–0008**

To Whom It May Concern:

We are writing on behalf of Human Rights Watch to submit the following comments and recommendations to the Consumer Financial Protection Bureau (CFPB), Department of Health and Human Services (HHS), and Department of the Treasury (Treasury) (collectively, the "agencies"), in response to their July 12, 2023 "Request for Information Regarding Medical Payment Products" (RFI).[1]

Human Rights Watch is an independent, international non-governmental organization that investigates and reports on rights violations in over 100 countries worldwide. We document and report on human rights abuses and direct advocacy toward governments, armed groups, and businesses, pressing them to change or enforce their laws, policies, and practices to respect, protect, and fulfil human rights for all.

In recent years, we have published several reports on economic barriers to accessing health care in the United States.[2] Our most recent report, "In Sheep's Clothing: United States' Poorly Regulated Nonprofit Hospitals Undermine Health Care Access," released in June 2023, documented how the US government's failure to adequately regulate private nonprofit hospitals and to provide quality hospital care alternatives undermines the human right to health.[3]

---

[1] Consumer Financial Protection Bureau, Centers for Medicare & Medicaid Services, and Department of the Treasury, "Request for Information Regarding Medical Payment Products," 88 Fed. Reg. 44281, 44281 (July 12, 2023), https://www.federalregister.gov/documents/2023/07/12/2023-14726/request-for-information-regarding-medical-payment-products.

[2] See, for example, Human Rights Watch, *In Sheep's Clothing: United States' Poorly Regulated Nonprofit Hospitals Undermine Health Care Access* (2023), https://www.hrw.org/report/2023/06/15/sheeps-clothing/united-states-poorly-regulated-nonprofit-hospitals-undermine; Human Rights Watch, *"If I'm Out of Insulin, I'm Going to Die": United States' Lack of Regulation Fuels Crisis of Unaffordable Insulin* (2022), https://www.hrw.org/report/2022/04/12/if-im-out-insulin-im-going-die/united-states-lack-regulation-fuels-crisis; Human Rights Watch, *"We Need Access": Ending Preventable Deaths from Cervical Cancer in Rural Georgia* (2022), https://www.hrw.org/report/2022/01/20/we-need-access/ending-preventable-deaths-cervical-cancer-rural-georgia.

[3] "US: Nonprofit Hospitals Chase Low-Income Patients on Debts: Weak Government Oversight Fueling Care Crisis," Human Rights Watch, June 15, 2023, https://www.hrw.org/news/2023/06/15/us-nonprofit-hospitals-chase-low-income-patients-debts.

Accordingly, Human Rights Watch strongly supports the agencies' effort to better regulate the rights-impacting practices of private entities involved in the healthcare system and in the collection of medical debt. However, our research has made clear that more closely aligning the US healthcare system with the country's international human rights obligations will require the agencies, acting on behalf of the US government, to address the structural drivers of medical debt, rather than just treating its symptoms.

As the agencies' RFI makes clear, the US healthcare system has persistent problems with both high out-of-pocket costs for accessing health care and the indebtedness that these costs create.[4] Our research has shown how, together, these form sometimes insurmountable barriers to accessing health care and adversely affect people's enjoyment of other human rights, including the rights to housing, food, and education.[5]

Hospital services, in particular, significantly contribute to the country's medical debt crisis, with a March 2023 survey conducted by the Urban Institute finding that about 73 percent of adults with past-due medical debt reported owing at least some of that debt to hospitals.[6] But nearly 60 percent of the more than 5,000 community hospitals across the US are privately operated nonprofits, which collectively receive public subsidies worth billions each year, largely in the form of state and federal tax exemptions, in exchange for providing "community benefits" like free or reduced-price health care.[7]

---

[4] Because of this absence of universally accessible and adequate health care for everyone, in 2021, about 20 percent of US adults experienced a major, unexpected medical expense that had a median out-of-pocket cost between $1,000 and $1,900. "Economic Well-Being of US Households in 2021," Board Of Governors of the Federal Reserve System, May 2022, p. 38, https://www.federalreserve.gov/publications/files/2021-report-economic-well-being-us-households-202205.pdf. As a result of these out-of-pocket costs, about 41 percent of US adults had some form of outstanding debt due to medical or dental bills in 2022, while 24 percent of adults were either past due or unable to pay their health care bills at the time of the were surveyed. Lunna Lopes, Audrey Kearney, Alex Montero, Liz Hamel, and Mollyann Brodie, "Health Care Debt in the U.S.: The Broad Consequences of Medical and Dental Bills," Kaiser Family Foundation, June 16, 2022, https://www.kff.org/reportsection/kff-health-care-debt-survey-main-findings/.

[5] See Note 2, above. See also Munira Z. Gunja, Evan D. Gumas, Reginald D. Williams II, "US Health Care from a Global Perspective, 2022: Accelerating Spending, Worsening Outcomes," The Commonwealth Fund, January 31, 2023, https://www.commonwealthfund.org/publications/issue-briefs/2023/jan/us-health-care-global-perspective-2022 (comparing rates of cost-related rationing and deferment of medical treatment in the US to other countries in the Organisation for Economic Co-operation and Development); David U. Himmelstein, Samuel L. Dickman, Danny McCormick, et al., "Prevalence and Risk Factors for Medical Debt and Subsequent Changes in Social Determinants of Health in the US," *Journal of the American Medical Association Network Open*, September 16, 2022, Vol. 5, No. 9, https://jamanetwork.com/journals/jamanetworkopen/fullarticle/2796358 (analyzing more than 142,000 survey results from the US Census Bureau and finding that people who acquired medical debt between 2017 and 2019 were about 2.2 times more likely to become food insecure and nearly 3 times more likely to experience an eviction or foreclosure).

[6] Michael Karpman, "Most Adults with Past-Due Medical Debt Owe Money to Hospitals," Urban Institute, March 13, 2023, https://www.urban.org/research/publication/most-adults-past-due-medical-debt-owe-money-hospitals. See also Consumer Financial Protection Bureau, "Medical Debt Burden in the United States," February 2022, https://files.consumerfinance.gov/f/documents/cfpb_medical-debt-burden-in-the-united-states_report_2022-03.pdf (discussing throughout how hospital practices contribute to medical debt in the US).

[7] The American Hospital Association's annual survey of community hospitals found that 2,960 of the 5,139 community hospitals across the US in 2022 were organized as not-for-profit entities. American Hospital Association, "Fast Facts on US Hospitals, 2022," January 2022, https://www.aha.org/statistics/fast-facts-us-hospitals. Although the Internal Revenue Service (IRS) considers a hospital's provision of charity care to be a significant factor indicating their "community benefit," qualifying for tax exemption under Rev. Rul. 69-545, 1969-2 C.B. 117 does not technically require this so-called "charity care." See US Internal Revenue Service, "Rev. Rul. 69-545, 1969-2 C.B. 117," 1969, https://www.irs.gov/pub/irs-tege/rr69-545.pdf; US Government Accountability Office, "Tax Administration: Opportunities Exist to Improve Oversight of Hospitals' Tax Exempt Status," September 2020, p. 7, https://www.gao.gov/assets/gao-20-679.pdf (accessed April 18, 2023); US Internal Revenue Service, "Charitable Hospitals - General Requirements for Tax-Exemption Under Section 501(c)(3)," July 15, 2022, https://www.irs.gov/charities-non-profits/charitable-hospitals-general-requirements-for-tax-exemption-under-section-501c3 ("Rev. Rul. 56-185, modified by Rev.

The US heavily relies on these privately operated nonprofit hospitals' so-called "charity care" to increase access to health care for patients who cannot pay for hospital services, particularly for the tens of millions of people in the US who are uninsured.[8] But given the high prevalence of hospital-related medical debt in the US, this charity-dependent model of hospital services is clearly not working.[9]

However, the agencies have the authority to initiate new rulemaking that can significantly improve this system, reducing the actual prevalence of — and not just the effects of — medical debt. Accordingly, while the RFI focuses primarily on medical payment products, we are encouraged by the agencies' call for additional information about the practices of these tax-exempt institutions.

Drawing from our research in the US, this submission responds to these calls for additional information, outlining debt-producing practices of tax-exempt hospitals that should be regulated to improve access to health care.

## I. "Treasury-Specific Questions" - II(d)(5)

**"What are best practices for hospitals publishing and making patients aware of financial assistance programs (beyond compliance with the widely publicized requirements found in the section 501(r) regulations)?"[10]**

Despite existing publication and notification requirements under section 501(r) of the Internal Revenue Code (IRC), nonprofit hospitals' charity care practices regularly fail to ensure that patients who should qualify for free or reduced-price care under their self-defined financial assistance policies actually receive this treatment.[11] Rather, as documented in our June 2023 report, these tax-exempt hospitals sometimes bill, and even sue, patients who should have been eligible for charity care.[12]

---

Rul. 69-545, set forth more restrictive requirements for exemption of hospitals under Section 501(c)(3)… Though a hospital is no longer required to operate under those requirements, doing so continues to be a significant factor indicating that the hospital is operated for the benefit of the community.").

[8] For more information on charity care requirements, see generally Congressional Research Service, "Hospital Charity Care and Related Reporting Requirements Under Medicare and the Internal Revenue Code," June 28, 2018, https://sgp.fas.org/crs/misc/IF10918.pdf. More than 8 percent of the US population, or approximately 30 million people, did not have any health insurance in 2021. Compare Katherine Keisler-Starkey and Lisa N. Bunch, "Health Insurance Coverage in the United States: 2021," US Census Bureau, September 14, 2021, https://www.census.gov/library/publications/2022/demo/p60-278.html ("In 2021, 8.3 percent of people, or 27.2 million, did not have health insurance at any point during the year") with "Health Insurance Coverage of the Total Population," Kaiser Family Foundation, https://www.kff.org/other/stateindicator/total-population (citing US Census Bureau, "American Community Survey," 2021, https://www.census.gov/programs-surveys/acs) (estimating that 8.6 percent of the US population was uninsured in 2023)). For more discussion about the lack of health insurance coverage in the US, see Human Rights Watch, *In Sheep's Clothing*, pp. 10-14.

[9] See Note 6, above.

[10] 88 Fed. Reg. at 44290.

[11] See "Requirements for 501(c)(3) Hospitals Under the Affordable Care Act – Section 501(r)," Internal Revenue Service, July 15, 2022, https://www.irs.gov/charities-non-profits/charitable-organizations/requirements-for-501c3-hospitals-under-the-affordable-care-act-section-501r.

[12] See Human Rights Watch, *In Sheep's Clothing,* pp. 41-55.

There are several reasons why patients who should be eligible for charity care may not receive it.[13] But research and investigations, much of which is compiled in our 2023 report on US nonprofit hospitals, indicates that many such exclusions are driven more by hospital practices than patient choices.[14]

Notably, many of these exclusion-producing hospital practices may already be prohibited under the agencies' existing regulations around financial assistance. But as a 2022 research brief by the CFPB found, there is "little evidence that compliance with financial assistance rules is systematically monitored and enforced at the federal level."[15] This conclusion is largely supported by a 2020 Government Accountability Office study, which similarly found lax Internal Revenue Service (IRS) oversight and enforcement of tax-exempt hospitals.[16]

Accordingly, if Treasury is interested in improving patients' access to tax-exempt hospitals' financial assistance, a first step would be to increase the auditing and enforcement of nonprofit hospitals' compliance with existing 501(r) obligations. Treasury should also strengthen and clarify these obligations, which are currently far too vague to adequately guide effective enforcement.[17]

As such, we urge Treasury to provide clearer guidance around tax-exempt hospitals' charity care obligations, including by specifying minimum financial eligibility criteria. We also recommend that Treasury amend the definition of "community benefit" by reinstating the previously explicit requirement that a nonprofit hospital "must be operated to the extent of its financial ability for those not able to pay for the services rendered."[18]

Treasury should also consider amending IRS rules that excuse any hospital's violation of section 501(r), so long as a correction is made and the violation was "neither willful nor egregious."[19] As it can be difficult to establish that a nonprofit hospital willfully intended to violate these rules when it, for example, sued patients who should have been eligible for charity care under its own policies, the ability of the IRS to penalize and deter violators is severely limited. As recommended in our 2023 report, we urge Treasury to change this standard to focus on the harm caused by these violations, rather than the intent of the violator.[20]

Treasury should also consider other changes to policy and practice that would better ensure nonprofit hospitals' compliance with charity care policies, including:

---

[13] See, for example, Andrea Bopp Stark and Jenifer Bosco, "An Ounce of Prevention: A Review of Hospital Financial Assistance Policies in the States," National Consumer Law Center, November 2021, https://www.nclc.org/wp-content/uploads/2022/09/Rpt_Ounce_of_Prevention.pdf.

[14] See Human Rights Watch, *In Sheep's Clothing,* pp. 41-55.

[15] Susan Singer, Eric Wilson and Tavi Carare, "Understanding Required Financial Assistance in Medical Care," US Consumer Financial Protection Bureau, July 28, 2022, https://www.consumerfinance.gov/data-research/research-reports/understanding-required-financial-assistance-in-medical-care/ (accessed April 18, 2023).

[16] US Government Accountability Office, "Tax Administration: Opportunities Exist to Improve Oversight of Hospitals' Tax-Exempt Status," September 2020, pp. 21-22, https://www.gao.gov/assets/gao-20-679.pdf,

[17] See Human Rights Watch, *In Sheep's Clothing,* pp. 22-26.

[18] Internal Revenue Service, "Rev. Ruling 56-185, 1956-1, CB 202," https://www.irs.gov/pub/irs-tege/rr56-185.pdf. Cf. Internal Revenue Service, "Rev. Rul. 69-545, 1969-2 C.B. 117," 1969, https://www.irs.gov/pub/irs-tege/rr69-545.pdf.

[19] 26 CFR § 1.501(r); ; "Billing and Collections – Section 501(r)(6)," US Internal Revenue Service, July 15, 2022, https://www.irs.gov/charities-non-profits/billing-and-collections-section-501r6.

[20] Human Rights Watch, *In Sheep's Clothing,* pp. 5-6.

- Requiring hospitals to use, and assist with the use of, a uniform and easily accessible financial assistance application, available in languages used by the communities served.
- Requiring nonprofit hospitals to submit their financial assistance policies within their community health needs assessments.
- Publicly disclosing section 501(r) violations and requiring that violating hospitals publish a public remediation plan.
- Prohibiting certain non-financial eligibility limitations, such as residency or lawful presence requirements, along with other barriers to providing assistance for people who are underinsured, including blanket bans against patients with insurance.

## II. "Treasury-Specific Questions" - II(d)(1)

**"What policy actions should Treasury consider taking to address problematic practices … including debt collection … to conform with the existing tax laws and regulations pertaining to tax-exempt hospitals?"[21]**

One of the requirements of IRC section 501(r) mandates that nonprofit hospitals make a "reasonable effort" to determine if a patient is eligible for charity care under the hospital's policy before engaging in so-called "extraordinary collection actions," such as initiating a legal or judicial process against them or selling their debts to third-party debt collectors.[22] As currently implemented, these extraordinary collection actions regulations are clearly failing to protect patients.

According to a nationally representative review of consumer credit reports conducted by the Urban Institute in 2022, about one in eight people in the US had medical debt in collections.[23] While it is unclear exactly how much of this medical debt in collections comes from nonprofit hospitals, which account for most community hospitals in the US, available research indicates that they are major contributors.[24]

For example, our 2023 report compiled media accounts and studies from across the US that documented tens of thousands of lawsuits brought by nonprofit hospitals against patients in recent years.[25]

As Human Rights Watch has documented, many courts across the US routinely award default judgments in these kinds of debt collection cases, issuing no-questions-asked judgments without requiring meaningful evidence.[26] As such, once a patients' debt enters this stage of collections, they may rapidly find themselves on the other side of a rubber-stamped legal judgment that can cause significant financial hardship.

---

[21] 88 Fed. Reg. at44290.

[22] "Billing and Collections – Section 501(r)(6)," US Internal Revenue Service, July 15, 2022, https://www.irs.gov/charities-non-profits/billing-and-collections-section-501r6.

[23] Urban Institute, "Debt in America: An Interactive Map," June 23, 2022, https://apps.urban.org/features/debt-interactive-map/?type=medical&variable=medcoll.

[24] For example, a 2021 study of nearly 20 years of court records in Wisconsin, conducted by researchers from Yale and Stanford University, found that nonprofit hospitals in the state were actually more likely to sue patients to collect on medical debt than for-profit hospitals over this period. Zack Cooper, James Han, and Neale Mahoney, "Hospital Lawsuits over Unpaid Bills Increased by 37 Percent in Wisconsin from 2001 to 2018," Health Affairs, December 2021, Vol. 40, No. 12, https://www.healthaffairs.org/doi/10.1377/hlthaff.2021.01130.

[25] See Human Rights Watch, *In Sheep's Clothing,* pp. 45-55

[26] Human Rights Watch, "Rubber Stamp Justice: US Courts, Debt Buying Corporations, and the Poor," January 20, 2016, https://www.hrw.org/node/285470/ ("Many individual courts issue thousands or even tens of thousands of no questions asked default judgments in favor of debt buyers every year").

Treasury can dramatically improve compliance with these existing extraordinary collection actions regulations by, as noted in the section above, increasing its auditing and oversight of nonprofit hospitals, which currently suffers from a deficit of enforcement activities.[27]

In addition to increased enforcement, Treasury should consider amending IRS rules to strengthen the definition of "reasonable efforts" under IRC section 501(r)(6).[28] For example, IRS could require tax-exempt hospitals to provide additional, specific evidentiary support proving that they have made a "reasonable effort," or may entirely shift the burden of this standard, requiring that tax-exempt hospitals make a reasonable effort to disprove a patients' presumed eligibility for financial assistance before engaging in extraordinary collection actions.

Treasury should also consider other changes to policy and practice that would better ensure nonprofit hospitals' compliance with extraordinary collection actions rules, including:

- Prohibiting tax-exempt hospitals from engaging in particularly rights-impacting practices relating to extraordinary collection actions, such as foreclosures, wage garnishments, bank account seizures, and the charging of interest on medical debt.
- Requiring that all parties involved in extraordinary collection actions for medical debts accrued at tax-exempt hospitals (e.g., third-party debt collectors) share the burden of proving their "reasonable efforts" to determine that an individual against whom they are seeking debts is not eligible for financial assistance under the hospital organization's charity care policies.
- Publicly disclosing section 501(r) violations and requiring that violating hospitals publish a public remediation plan.

## III. HHS-Specific Questions

**"How might HHS improve patient understanding of options for covering the cost of medical treatments? At what points in the care process could patients be provided with information about their financial obligations and payment options?"[29]**

In the context of hospital services, the most fundamental way that HHS can improve patients' understanding of options for covering the cost of their medical treatment would be for it to use its authority to increase the accessibility of hospital financial assistance.

In particular, the Centers for Medicare and Medicaid Services (CMS), within HHS, should increase enforcement of the requirement that hospitals "provide all or a portion of services free of charge to patients who meet the hospital's charity care policy."[30]

---

[27] See Notes 15-16, above.
[28] Billing and Collections – Section 501(r)(6)," US Internal Revenue Service, July 15, 2022, https://www.irs.gov/charities-non-profits/billing-and-collections-section-501r6.
[29] 88 Fed. Reg. at 44289.
[30] Congressional Research Service, "Hospital Charity Care and Related Reporting Requirements Under Medicare and the Internal Revenue Code," June 28, 2018, https://sgp.fas.org/crs/misc/IF10918.pdf.

CMS should also require that hospitals comply with relevant consumer financial protection laws, such as the Dodd-Frank Wall Street Reform and Consumer Protection Act and IRC section 501(r), within the definition of "applicable Federal laws" under 42 CFR § 482.11(a), in order to receive compensation from CMS under the Conditions of Participation and Conditions for Coverage to continue participating in the Medicare and Medicaid programs. CMS could also extend these Conditions of Participation to prohibit particularly rights-impacting practices relating to extraordinary collection actions, such as foreclosures, wage garnishments, bank account seizures, and the charging of interest on medical debt.

## IV. Conclusion

Under international human rights law, everyone has the right to enjoy the highest attainable standard of health.[31] To realize this right, governments should ensure equal access to quality health care for all, regardless of a person's ability to pay.[32] The agencies, as agents of the US government, share its obligations to respect, protect, and fulfil this and all other human rights.[33]

Our research in the US has made clear that the federal government's failure to adequately regulate private nonprofit hospitals allows many to operate more like for-profit corporations, charging high fees and aggressively pursuing medical bills against people who cannot pay, including through lawsuits and selling debt to third-party debt collectors.[34] As a result of this inadequate regulation, unscrupulous nonprofit hospitals can hide among their responsible peers, benefiting from their public perception and tax-status as charitable institutions while engaging in extractive and exploitative practices that undermine human rights.[35]

We urge the agencies to make clear in their words and actions that health care is a right, not a privilege, reflecting both domestic and international US engagements. In the short term, we encourage the agencies to do this by enacting new rulemaking to ensure, at a minimum, that nonprofit hospitals' charity care is, in practice, easily obtainable, and adequately inclusive and robust to reflect community need.

Thank you for this opportunity to provide comment. We would be happy to provide additional information, and also welcome the opportunity to further discuss these issues and our research. If you would like to arrange such a meeting, please contact Matt McConnell (mcconnm@hrw.org) and Jack Spehn (spehnj@hrw.org).

**Matt McConnell**

Researcher
Economic Justice and Rights
Human Rights Watch

**Jack Spehn**

Associate
Economic Justice and Rights
Human Rights Watch

[31] UN Committee on Economic, Social and Cultural Rights, General Comment No. 14, The Right to the Highest Attainable Standard of Health, UN Doc. E/C.12/2000/4, August 11, 2000, para. 1, https://www.refworld.org/pdfid/4538838d0.pdf.
[32] Ibid.
[33] Ibid.
[34] Human Rights Watch, *In Sheep's Clothing*, pp. 29-37, 41-55.
[35] Ibid, pp. 45-55.

## V. Appendix

**Summary of Recommendations for Treasury:**

- Increase the auditing and enforcement of nonprofit hospitals' compliance with existing 501(r) obligations.
- Strengthen and clarify nonprofit hospitals' 501(r) obligations, which are currently far too vague to adequately guide effective enforcement, including by specifying minimum financial eligibility criteria.
- Strengthen requirements for satisfying the "community benefit" standard by reinstating the previously explicit requirement that a nonprofit hospital "must be operated to the extent of its financial ability for those not able to pay for the services rendered."
- Amend IRS rules that excuse any hospital's violation of section 501(r), so long as a correction is made and the violation was "neither willful nor egregious," to focus on the harm caused by these violations, rather than the intent of the violator.
- Strengthen the definition of "reasonable efforts" under IRC section 501(r)(6).
- Require tax-exempt hospitals to use, and assist with the use of, a uniform and easily accessible financial assistance application, available in languages used by the communities served.
- Require tax-exempt hospitals to submit their financial assistance policies within their community health needs assessments.
- Publicly disclose section 501(r) violations and require that violating hospitals publish a public remediation plan.
- Prohibit tax-exempt hospitals from engaging in certain non-financial eligibility limitations, such as residency or lawful presence requirements, along with other barriers to providing assistance for people who are underinsured, including blanket bans against patients with insurance.
- Prohibit tax-exempt hospitals from engaging in particularly rights-impacting practices relating to extraordinary collection actions, such as foreclosures, wage garnishments, bank account seizures, and the charging of interest on medical debt.
- Require that all parties involved in extraordinary collection actions for medical debts accrued at tax-exempt hospitals (e.g., third-party debt collectors) share the burden of proving their "reasonable efforts" to determine that an individual against whom they are seeking debts is not eligible for financial assistance under the hospital organization's charity care policies.

**Summary of Recommendations for HHS:**

- Increase enforcement of CMS' requirement that hospitals "provide all or a portion of services free of charge to patients who meet the hospital's charity care policy."
- Amend CMS' Conditions of Participation and Conditions for Coverage to require compliance with IRC section 501(r) and other relevant consumer financial protection laws, such as the Dodd-Frank Wall Street Reform and Consumer Protection Act, within the definition of "applicable Federal laws" under 42 CFR § 482.11(a).
- CMS could also extend these Conditions of Participation to prohibit particularly rights-impacting practices relating to extraordinary collection actions, such as foreclosures, wage garnishments, bank account seizures, and the charging of interest on medical debt.